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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-6
Patient Episode	: H19000054474	**Collection Date :**	09 Oct 2026 11:01
Referred By	: HEALTH CHECK MHH	**Reporting Date :**	09 Oct 2026 12:02
Receiving Date	: 09 Oct 2026 11:02		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
PLASMA GLUCOSE - PP			**Specimen:Plasma**
Plasma GLUCOSE - PP	**156 #**	**mg/dL**	**[70-140]**

Method: Hexokinase

Note : Conditions which can lead to lower postprandial glucose levels as compared to fasting glucose are excessive insulin release, rapid gastric emptying, brisk glucose absorption , post exercise

Glucose measurements are used in the diagnosis of carbohydrate metabolism disorders includin diabetes mellitus, neonatal hypoglycemia, and pancreatic islet cell carcinoma.

----------**END OF REPORT**----------

M. Arulkumar

Dr Arulkumar M
PhD Consultant Biochemist

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-4
Patient Episode	: H19000054474	**Collection Date :**	09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date :**	09 Oct 2026 10:30
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Specimen: serum **VITAMIN B12**	109 #	pg/ml	[222-1439]
Method: Chemiluminescence			

----------END OF REPORT----------

M. Arulkumar

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-2
Patient Episode	: H19000054474	**Collection Date**	: 09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date**	: 09 Oct 2026 10:25
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Specimen : Serum			
Serum TSH (CHEMILUMINESCENCE)	2.700	µIU/mL	[0.380-5.330]

TSH levels are increased in primary hypothyroidism, insufficient thyroid hormone replacement therapy, Hashimoto thyroiditis, use of amphetamines, dopamine antagonists, iodine containing agents, lithium and iodine induced or deficiency goiter.
Decreased levels of TSH may be seen in Grave's disease, toxic multinodular goiter, thyroidit excessive treatment with thyroid hormone replacement and central hypothyroidism

Specimen : Serum			
SERUM URIC ACID (Uricase)	6.77	mg/dL	[3.50-7.20]

Serum LIVER FUNCTION TEST			**Specimen: Serum**
TOTAL PROTEIN (Biuret)	7.40	g/dL	[6.60-8.30]
SERUM ALBUMIN (Bromocresol Green)	4.60	g/dL	[3.50-4.80]
SERUM GLOBULIN (Calculated)	2.80	g/dL	[2.50-3.50]
ALB/GLOB (A/G) Ratio	1.6		[1.0-2.5]
BILIRUBIN-TOTAL (DPD)	0.67	mg/dL	[0.10-1.20]
BILIRUBIN - DIRECT (DIAZO)	0.13	mg/dL	[0.10-0.50]
BILIRUBIN - INDIRECT (CALCULATED)	0.54	mg/dL	[0.20-0.70]
SGOT/ AST (Enzymatic UV without P5P)	21	U/L	[15-41]
SGPT/ ALT (Enzymatic UV without P5P)	26	U/L	[17-63]
ALP (PNP - AMP)	71	U/L	[30-120]
GGT	26	U/L	[7-50]

Method:Gamma glutamyl carboxy nitroanilide

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-2
Patient Episode	: H19000054474	**Collection Date :**	09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date :**	09 Oct 2026 10:25
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
PROSTATE SPECIFIC ANTIGEN			
Specimen: Serum			
TOTAL PSA (PROSTATE - SPECIFIC ANTIGEN)	0.330	ng/ml	[0.000-4.000]
Method: chemiluminescence			

Note : PSA is a glycoprotein that is produced by the prostate gland. Normally, very little PSA is secreted in the blood. Increases in glandular size and tissue damage caused by BPH, prostatitis, or prostate cancer may increase circulating PSA levels.

Caution : Serum markers are not specific for malignancy, and values may vary by method.

Immediate PSA testing following digital rectal examination, ejaculation, prostate massage urethral instrumentation, prostate biopsy may increase PSA levels.

Some patients who have been exposed to animal antigens, may have circulating anti-animal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

----------**END OF REPORT**----------

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-3
Patient Episode	: H19000054474	**Collection Date :**	09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date :**	09 Oct 2026 10:25
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
HbA1c (Glycosylated Hemoglobin)	5.8 #	%	[<5.7]
Estimated Average Glucose(eAG)	120	mg/dl	

SPECIMEN : WHOLE BLOOD
Method : Ion Exchange HPLC

Biological reference interval:
Normal: < or = 5.6 %,
Increased risk for Diabetes (Pre-diabetes): 5.7 - 6.4 %,
Diabetes: > or = 6.5 %,
If you are a Diabetic, please consult your doctor for your HbA1c target.

----------END OF REPORT----------

M. Arulkumar

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-2
Patient Episode	: H19000054474	**Collection Date :**	09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date :**	09 Oct 2026 10:21
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
GLUCOSE-Fasting	**Specimen: Serum**		
GLUCOSE-Fasting Method:Hexokinase G6PDH / UV	93	mg/dL	[<100]

Glucose measurements are used in the diagnosis of carbohydrate metabolism disorders includin diabetes mellitus, neonatal hypoglycemia, and pancreatic islet cell carcinoma.

SERUM CREATININE METHOD: Creatininase/creatinase/sarcosineoxidase/POD	**0.77 #**	**mg/dL**	**[0.90-1.30]**
Specimen: Serum Blood Urea Nitrogen(Calculated)	8.27	mg/dL	[8.00-23.00]
Calcium Total	**Specimen- Serum**		
SERUM CALCIUM (Arsenazo III)	10.03	mg/dL	[8.90-10.30]

Hypercalcemia (increased value of calcium) can be caused by increased intestinal absorption (vitamin D intoxication), increased skeletal reabsorption (immobilization), or a combination mechanisms (primary hyperparathyroidism). Primary hyperparathyroidism and malignancy accoun 90-95% of all cases of hypercalcemia. Values of total calcium is affected by serum protein particularly albumin thus, latter's value should be taken into account when interpreting ser calcium levels.

Common causes of decreased value of calcium (hypocalcaemia) are chronic renal failure, hypomagnesaemia and hypoalbuminemia.

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LIFE'S ON

MG-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-2
Patient Episode	: H19000054474	**Collection Date**	: 09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date**	: 09 Oct 2026 10:21
Receiving Date	: 09 Oct 2026 08:19		

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
LIPID PROFILE			Specimen: Serum
TOTAL CHOLESTEROL (CHOD/POD)	**247 #**	**mg/dL**	**[<200]Normal:**
Method: ENZYMATIC,CO,CE, Peroxidase			Moderate risk:200-239
			High risk:>240
TRIGLYCERIDES(Triglycerides)	**177 #**	**mg/dL**	**[<150]Normal:**
Method: GPO TRINDER GLYCEROL BLANK			Borderline high:151-199
			High: 200 - 499
			Very high:>500
HDL-CHOLESTEROL	57	mg/dl	[40-70]
(Method:Direct measure, immunoinhibition)			
VLDL-Cholesterol (Calculated)	35	mg/dL	[10-40]
LDL- CHOLESTEROL	**160 #**	**mg/dL**	**[<100]Normal:**
Method: Colorimetric,Direct Measure			Near or Above optimal:100-129
			Borderline High:130-159
			High Risk:160-189
T.CHOL/HDL.CHOL Ratio	4.3		<4.0 Optimal
Method: Calculated			4.0-5.0 Borderline
			>6 High Risk
LDL.CHOL/HDL.CHOL Ratio	2.8		<3 Optimal
Method: Calculated			3-4 Borderline
			>6 High Risk
HDL:LDL RATIO	0.4		
Method: Calculated			

----------END OF REPORT----------

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LIFE'S ON

MC-2397

Name	: MR JAWAHARLAL NEHRU E	**Age**	: 37 Yr(s) Sex :Male
Registration No	: MH019187481	**Lab No**	: 2605720982-1
Patient Episode	: H19000054474	**Collection Date**	: 09 Oct 2026 08:14
Referred By	: HEALTH CHECK MHH	**Reporting Date**	: 09 Oct 2026 09:43
Receiving Date	: 09 Oct 2026 08:19		

HAEMATOLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
COMPLETE BLOOD COUNT (AUTOMATED)		**SPECIMEN-EDTA Whole Blood**	
RBC COUNT (IMPEDENCE)	5.43	millions/cumm	[4.50-5.50]
HEMOGLOBIN (CYANOMETHEMOGLOBIN)	15.5	g/dl	[13.0-17.0]
HEMATOCRIT (CALCULATED)	44.0	%	[40.0-50.0]
MCV (DERIVED)	**81.1 #**	**fL**	**[83.0-101.0]**
MCH(CALCULATION METHOD)	28.5	pg	[27.0-32.0]
MCHC(CALCULATION METHOD)	**35.2 #**	**g/dl**	**[31.5-34.5]**
RDW CV% (DERIVED)	13.2	%	[11.6-14.0]
PLATELET COUNT(IMPEDENCE)	298	x 10^3 cells/cumm	[150-410]
Total Leucocyte Count(IMPEDENCE)	7.30	x 10^3 cells/cumm	[4.00-10.00]
DIFFERENTIAL COUNT (VCS TECHNOLOGY)			
Neutrophils	53.9	%	[40.0-80.0]
Lymphocytes	33.8	%	[20.0-40.0]
Monocytes	7.4	%	[2.0-10.0]
Eosinophils	4.2	%	[1.0-6.0]
Basophils	0.7	%	[0.0-2.0]

----------**END OF REPORT**----------

Dr. Yashaswini R
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LIFE'S ON

Name	:	MR JAWAHARLAL NEHRU E	**Age**	:	37 Yr(s) Sex :Male
Registration No	:	MH019187481	**Lab No**	:	2605720982-2
Patient Episode	:	H19000054474	**Collection Date :**		09 Oct 2026 08:14
Referred By	:	HEALTH CHECK MHH	**Reporting Date :**		09 Oct 2026 10:25
Receiving Date	:	09 Oct 2026 08:19			

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
VITAMIN D, 25 - HYDROXY			
VITAMIN D, 25 - HYDROXY	53.4	nmol/L	[50.0-250.0]
Specimen: Serum			
Reference range:			Deficient:- <50 nmol/L, Sufficient:- 75 - 250 nmol/L, Insufficient:- 50 - 75 nmol/L, Potential intoxication:- >250 nmol/L

----------END OF REPORT----------

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